SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Darolutamide: high-risk nonmetastatic castration-resistant prostate cancer

High-risk nonmetastatic castration-resistant prostate cancer · NCCP 00693 · version 2 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to control prostate cancer and delay progression. The expected benefit depends on your disease setting, previous treatment and general health.

Schedule
Darolutamide 600 mg twice daily with food, continuously while helping and tolerated. Continue ADT unless surgically castrated. The team may prescribe 300 mg twice daily for selected organ impairment or toxicity.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Darolutamide

ANDROGEN PATHWAY TREATMENT

- ☐ Tiredness and weakness
- ☐ Rash, limb pain and changes in blood counts or liver blood tests
- ☐ Ischaemic heart disease or heart failure
- ☐ Rare serious liver injury
- ☐ Interactions, including increased exposure to some cholesterol medicines
- ☐ Hot flushes, sexual or fertility effects and bone loss with ongoing ADT

Discuss the treatment aim, setting-specific evidence, alternatives and declining treatment; oral medicine and ADT risks, bone and cardiovascular health, sexual function, fertility, contraception, interactions, required tests and urgent contacts. Ask pharmacy to check all medicines, especially cholesterol medicines such as rosuvastatin and enzyme-inducing medicines or herbal products. Report jaundice, dark urine, severe tiredness, chest symptoms or a marked change in wellbeing promptly.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00693 v2.

SACTCheck v0.84.51
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